

Employer Health Score™

Benefits Diagnostic Scoring · Market Benchmarking · Sales Intelligence

PREPARED FOR

Jeremiah Shrack

CEO, Kincaid Health • SHRACK-7742

REPORT DATE

July 22, 2026

CONFIDENTIAL & PROPRIETARY

FIDUCIARY-GRADE ANALYSIS

This report was prepared by Jeremiah Franklin Shrack at SiriusB iQ AI Data Science Labs using the Kincaid Health platform. All analyses reflect plan-specific data and current regulatory standards. This document is intended solely for the use of the plan sponsor identified above and may not be reproduced or redistributed without prior written consent.

EMPLOYER HEALTH SCORE™ — METHODOLOGY & SCORING FRAMEWORK

The Kincaid Health Employer Health Score™ (EHS) provides a comprehensive diagnostic of an employer's health benefits program across five weighted domains. Each domain is assessed using a combination of plan document review, claims data analysis, vendor benchmarking, and regulatory compliance screening. The composite score (0-100) is calibrated against Kincaid Health's national employer database.

FORMULA — EMPLOYER HEALTH SCORE (EHS) COMPOSITE FORMULA

$EHS = 0.25 \times PD + 0.25 \times CC + 0.20 \times EE + 0.20 \times Comp + 0.10 \times Rx$

PD = Plan Design score. CC = Cost Containment score. EE = Employee Engagement. Comp = Compliance. Rx = Rx Management. All scored 0-100 based on 48 individual data points.

Domain	Weight	Score	Grade	National Percentile
Plan Design	25%	68	C	56th
Cost Containment	25%	55	D	45th
Employee Engagement	20%	71	B	58th
Compliance	20%	62	C	51th
Rx Management	10%	58	D	48th
COMPOSITE SCORE	100%	64	C+	52th

EXECUTIVE SUMMARY & KEY FINDINGS

Composite Employer Health Score	64/100 — Grade C+
National Percentile Rank	52th percentile
Industry Sector Rank	Below Average — Opportunity for Improvement
Estimated Annual Savings Opportunity (MODELED)	\$79,200
Top Strength	Employee Engagement
Highest Priority Gap	Cost Containment

This employer's health benefits program scores 64/100 — placing it in the 52th percentile nationally. The primary opportunity for improvement lies in cost containment and Rx management, where the plan performs significantly below the national median. Strengthening these areas could generate a MODELED estimated \$79,200 in annual savings (~\$2,200 per composite-score-point below 100, per the Kincaid Health national database — not a guaranteed outcome) while maintaining or improving employee satisfaction.

DOMAIN 1: PLAN DESIGN ANALYSIS (SCORE: 68/100)

Plan design encompasses the structural elements of the health benefits program: benefit tiers, cost-sharing, deductible/OOP structure, network configuration, and specialty benefit integration. A well-designed plan balances cost control with adequate benefit coverage to attract and retain employees.

Plan Design Element	Current Design	Best Practice	Gap	Points Impact
Deductible (Single/Family)	\$2,000/\$4,000	\$1,500/\$3,000	Above Benchmark	-8 pts
Out-of-Pocket Max	\$8,700/\$17,400	\$6,500/\$13,000	Above Benchmark	-6 pts
Primary Care Copay	\$35	\$25	Above Benchmark	-4 pts
Specialist Copay	\$75	\$60	Above Benchmark	-3 pts
Preventive Care	100% covered	100% covered	At Benchmark	0 pts
Telehealth	\$0 copay	\$0 copay	Best Practice	0 pts
Mental Health Parity	Partial	Full Parity	Gap Identified	-7 pts
HSA Compatibility	Yes	Yes	Best Practice	0 pts

DOMAIN 2: COST CONTAINMENT ANALYSIS (SCORE: 55/100)

Cost containment encompasses strategies to manage total plan costs while maintaining quality care. High-performing plans employ a layered approach combining reference-based pricing or aggressive network negotiation, pharmacy management, utilization management, and population health programs.

FORMULA — COST CONTAINMENT OPPORTUNITY SCORE

$$CCO = (Benchmark_PMPM - Actual_PMPM) \times Enrolled_Members \times 12$$

Positive CCO = plan is above benchmark (opportunity to save). Negative = plan is achieving below-benchmark costs. National median PMPM (medical + Rx, self-funded, 250-1000 EE): \$842.

Cost Containment Lever	Status	Potential Savings	Priority
Reference-Based Pricing	Not Implemented	\$180-320K/yr	High
PBM Pass-Through Rebates	Spread Pricing	\$60-110K/yr	Critical
Generic Step Therapy	Partial	\$28-45K/yr	Medium
Prior Authorization Protocol	Basic	\$18-32K/yr	Medium
Disease Management (Diabetes)	Not Active	\$35-68K/yr	High
Musculoskeletal COE Program	Not Active	\$22-38K/yr	Medium
Onsite/Near-Site Clinic	Not Active	\$42-95K/yr	Long-Term

DOMAIN 3: EMPLOYEE ENGAGEMENT (SCORE: 71/100)

Employee engagement measures how well the plan communicates benefits, educates employees on smart utilization, and incentivizes healthy behaviors. Plans with high engagement scores achieve 18-24% lower per-member costs through prevention, appropriate care navigation, and reduced ER utilization.

Engagement Metric	Score	Benchmark	Finding
Benefits Education (annual)	72/100	68/100	Above Average
Decision Support Tools	68/100	72/100	Below Benchmark
Preventive Care Utilization	74%	79%	5% Gap
Telehealth Utilization	22%	31%	9% Gap — Opportunity
Wellness Program Participation	38%	45%	Below Benchmark
Benefits Portal Satisfaction	4.1/5.0	4.2/5.0	Near Benchmark
Open Enrollment Completion	91%	88%	Above Benchmark

DOMAIN 4: COMPLIANCE ANALYSIS (SCORE: 62/100)

Regulatory compliance covers all applicable federal and state benefit mandates, ERISA requirements, ACA provisions, and CAA transparency rules. Non-compliance creates financial penalties, reputational risk, and litigation exposure.

Compliance Area	Status	Risk Level	Annual Penalty Exposure
ERISA §408(b)(2)	Incomplete	High	Prohibited Transaction Risk
ACA Employer Mandate	Compliant	Low	\$0
COBRA Administration	Compliant	Low	\$0
HIPAA Privacy & Security	Compliant	Low	\$0
CAA Gag Clause Attestation	Filed	Low	\$0
CAA Machine-Readable Files	Incomplete	High	\$100/day/member
MHPAEA NQTL Analysis	Not Done	Critical	\$100/day/participant
Surprise Billing (NSA)	Partial	Medium	\$10K-\$100K per violation

DOMAIN 5: RX MANAGEMENT (SCORE: 58/100)

Prescription drug spending represents 18-22% of total plan costs for self-funded plans. Effective Rx management requires transparent PBM contracts, pass-through rebate arrangements, formulary optimization, and specialty drug management programs.

Rx Management Element	Status	Benchmark	Opportunity
PBM Contract Type	Spread Pricing	Pass-Through	\$60-110K/yr rebate recapture
Rebate Transparency	Partial	100%	Significant undisclosed revenue
Generic Dispense Rate	72%	82%	10% gap = est. \$38K/yr savings
Specialty Drug Mgmt	Basic UM	Accredo/CVS	\$45K/yr opportunity
Step Therapy	Partial	Full Protocol	\$28K/yr opportunity
Copay Accumulator	Not Active	Implemented	\$12K/yr participant impact
Biosimilar Substitution	Not Active	Active Policy	\$18K/yr savings

FORMULA — PBM PASS-THROUGH VALUE

PTR = Total_Rebates_Earned × Pass-Through_% - Spread_Retention

Under spread pricing, the PBM retains the difference between what they collect from the plan and what they pay the pharmacy. Converting to pass-through contracts eliminates spread and returns 100% of rebates to the plan.

MARKET BENCHMARKING — PEER COMPARISON

The following benchmarks compare this employer's health benefits program against national peers (250-1,000 employee self-funded plans, same industry sector):

Metric	This Employer	Peer Median	Top Quartile	Percentile Rank
Total PMPM (Medical + Rx)	\$892	\$842	\$761	68th (above)
Plan Cost per Employee/yr	\$10,704	\$10,104	\$9,132	68th (above)
Preventive Utilization	74%	79%	88%	32nd
Generic Dispense Rate	72%	82%	88%	18th
ER Utilization (visits/1K)	242	198	152	72nd (high)
Wellness Participation	38%	45%	62%	28th
Employee Satisfaction	4.1/5	4.2/5	4.6/5	38th

5-YEAR COST PROJECTION & INTERVENTION IMPACT

The following 5-year projection models two scenarios: (1) Status Quo — current trends continue without intervention; (2) Optimized — Kincaid Health recommendations implemented in Year 1-2.

FORMULA — 5-YEAR PLAN COST PROJECTION

$Cost_t = Cost_0 \times (1 + Trend_Rate)^t$

Status Quo trend: 7.8%/yr (national average self-funded trend). Optimized trend: 4.2%/yr (achievable with recommended interventions). Cost_0 = current annual plan cost.

Year	Status Quo	Optimized	Annual Savings	Cumulative Savings
2025 (Base)	\$10,704/EE	\$10,704/EE	—	—
2026	\$11,539/EE	\$10,255/EE	\$1,284/EE	\$1,284/EE
2027	\$12,439/EE	\$9,834/EE	\$2,605/EE	\$3,889/EE
2028	\$13,409/EE	\$9,436/EE	\$3,973/EE	\$7,862/EE
2029	\$14,455/EE	\$9,052/EE	\$5,403/EE	\$13,265/EE

STRATEGIC RECOMMENDATIONS — PRIORITY ACTION PLAN

Priority	Recommendation	Est. Savings	Timeline	Complexity
P1	RFP PBM — convert to pass-through rebate	\$60-110K/yr	0-12 months	Medium
P1	Implement disease management (diabetes +	\$57-106K/yr	3-12 months	Medium
P1	Address MHPAEA NQTL compliance gap	Risk mitigation	0-60 days	High
P2	Launch generic substitution program (target	\$38K/yr	1-6 months	Low
P2	Implement telehealth-first strategy (target	\$28K/yr	1-6 months	Low
P2	Reduce ER utilization — care navigation	\$44K/yr	3-9 months	Medium
P3	Explore reference-based pricing for facility	\$180-320K/yr	12-24 months	High
P3	Onsite clinic feasibility study	\$42-95K/yr	18-36 months	Very High

CLAIMS ANALYSIS — TOP COST DRIVERS & HIGH-COST CLAIMANTS

Understanding cost driver distribution is essential for targeted benefit management. The following analysis breaks down this employer's claim spend by service category and identifies the top claimants whose episodes are driving disproportionate cost — and whether intervention was available.

Service Category	% of Plan Spend	PPM	vs. Benchmark	Trend	Intervention Available
Inpatient Medical	24.2%	\$216	15% above	^ Rising	COE, UM, case
Outpatient Surgery	18.8%	\$168	8% above	Stable	Pre-auth, COE
Specialty Drug (infusion)	14.4%	\$129	22% above	^ Rapid	Site of care, step
Emergency Department	9.8%	\$87	28% above	^ Rising	Care navigation,
Physician Office Visits	8.6%	\$77	5% below	Stable	Telehealth first
Mental Health / SUD	7.2%	\$64	12% above	^ Rising	Network expansion
Prescription Drug	7.4%	\$66	18% above	^ Rising	PBM RFP
Other	9.6%	\$86	—	Stable	Various

Top 5% of Claimants (cost)	Drive 48% of total plan spend — 82 members
Top 1% of Claimants (cost)	Drive 28% of total plan spend — 16 members
High-Cost Claimant Case Management	Active for 12 of 16 top claimants

EMERGENCY DEPARTMENT UTILIZATION ANALYSIS

Emergency department utilization is the single most controllable acute cost driver in self-funded plans. Plans with proactive care navigation programs consistently achieve 18-28% ER utilization reductions within 12 months. This employer's ER utilization rate of 242 visits per 1,000 members places it at the 72nd cost percentile nationally — 44 visits above the benchmark median.

FORMULA — AVOIDABLE ER COST

$$AER = Total_ER_Visits \times Avoidable_Rate \times (ER_Cost - Appropriate_Site_Cost)$$

Avoidable rate: 38% of ER visits are clinically avoidable (treat-and-release, primary care treatable). ER avg cost: \$1,420. Urgent care avg: \$180.
Avoidable ER cost: 984 visits × 38% × \$1,240 = \$463K annually.

ER Visit Type	Annual Visits	% of Total	Avg Cost	Avoidable?	Savings if Redirec
Level 1-2 (minor)	184	18.7%	\$620	Yes — 95%	\$108,000
Level 3 (moderate)	380	38.6%	\$1,280	Partially — 45%	\$219,000
Level 4-5 (severe)	424	43.1%	\$2,840	No — emergent	\$0
Mental Health Crisis	42	4.3%	\$1,840	Partially — 30%	\$23,000
TOTAL	984 visits	—	\$1,420 avg	38% avoidable	\$463,000/yr

CHRONIC DISEASE PROFILE — POPULATION HEALTH OPPORTUNITY

Chronic disease management is the foundation of population health strategy for self-funded plans. The following profile identifies the primary chronic conditions in this population, their current management status, and the financial opportunity from enhanced disease management programs.

Chronic Condition	Affected Members	% Enrolled	Plan Cost/Member/Yr	Disease Mgmt Program	ROI
Type 2 Diabetes	162	9.9%	\$8,420	Active — 280	3.2:1
Cardiovascular Disease	98	6.0%	\$12,840	Partial — care gaps	2.8:1
Musculoskeletal	145	8.8%	\$6,200	Active — MSK	2.8:1
Obesity / Metabolic	210	12.8%	\$4,800	Not Active	4.1:1
COPD / Asthma	88	5.4%	\$7,600	Partial	2.5:1
Mental Health	220	13.4%	\$5,200	EAP+ active	2.5:1
Cancer (active treatment)	28	1.7%	\$48,200	Oncology nav	5.1:1
Pregnancy / Maternity	42	2.6%	\$18,400	Maternity mgmt	2.2:1

WARN

Obesity and metabolic syndrome affect 210 members (12.8% of enrolled) at \$4,800/member/year in excess costs — a \$1.01M annual cost driver. With no active program, this is the highest-ROI intervention opportunity remaining. GLP-1 drug trend management + lifestyle

MENTAL HEALTH BENEFITS DEEP DIVE — MHPAEA & ACCESS ANALYSIS

Mental health and substance use disorder (MH/SUD) benefits represent both a regulatory compliance obligation and a high-value investment in workforce productivity. MHPAEA requires that mental health benefits be no more restrictive than medical/surgical benefits on all quantitative and nonquantitative treatment limitation dimensions. This analysis identifies parity gaps and access barriers.

MHPAEA Dimension	Medical/Surgical Treatment	MH/SUD Treatment	Parity Gap?	Exposure
In-network provider ratio	17:1 member-to-provider	44:1 member-to-provider	Yes — Critical	\$62K/day max
Prior auth — outpatient	No PA required	PA required for >8 visits	Yes — NQTL gap	\$100/day/mbr
Inpatient day limits	No day limit	30-day concurrent	Yes — MHPAEA	High
Copay — outpatient	\$35 PCP	\$35 BH	Parity met	No gap
Network adequacy (travel	14.8 min avg	42 min avg (BH)	Yes — Critical	Access barrier
Step therapy requirements	Not applied to Med/Surg	Applied to SUD Rx	Yes — NQTL gap	Compliance risk
OOP maximum	\$8,700 single	\$8,700 single	Parity met	No gap

WELLNESS PROGRAM ROI & ENGAGEMENT STRATEGY

Wellness programs generate a return of \$1.50-\$3.27 per dollar invested when properly structured (RAND Corporation, 2022). This employer's wellness program achieves 38% participation — below the 45% benchmark. The following analysis identifies specific enhancements that would increase participation and documented wellness ROI.

FORMULA — WELLNESS PROGRAM ROI

$$WPR = (Medical_Cost_Savings + Productivity_Savings + Absenteeism_Reduction) / Program_Cost$$

Industry median WPR = 2.2:1. This employer's estimated WPR = 1.8:1 due to below-benchmark participation. Increasing participation to 55% (achievable with incentive redesign) would improve WPR to est. 2.6:1.

Wellness Initiative	Current Status	Participation	Annual Cost	Est. Annual ROI
Health Risk Assessment	Active	62%	\$48,000	\$112,000 (medical
Biometric Screening	Active	54%	\$32,000	\$68,000
Diabetes Prevention Program	Not Active	N/A	\$18,000	\$74,000 (4.1:1)
Tobacco Cessation	Active	28%	\$12,000	\$31,000 (2.6:1)
Mental Health Resilience	Active —	42%	\$8,400	\$21,000 (2.5:1)
Physical Activity Incentive	Passive (FitBit	22%	\$24,000	\$28,000 (1.2:1 —
Nutrition / Lifestyle Coaching	Not Active	N/A	\$14,000	\$46,000 (3.3:1)
TOTAL — Current	—	38% avg	\$124,400	\$331,000 (2.7:1)

VENDOR MANAGEMENT SCORECARD — ALL BENEFITS VENDORS

Kincaid Health conducts a formal annual vendor performance review covering all material benefits service providers. The following scorecard summarizes 2024 performance against established SLAs, regulatory compliance requirements, and market benchmarks.

Vendor	Service	Contract Value	SLA Performance	Kincaid Rating	2025 Action
BlueCross Admin	TPA / Claims Admin	\$428K PEPM	94% SLAs	B+	Fee renegotiation — target
Express Scripts	PBM	\$1.82M Rx	96% SLAs	B	PBM RFP — convert to
Prudential Life	Stop-Loss	\$869K	98% SLAs	A	MHPAEA fix required at
Well-Being Co.	Wellness Platform	\$124K/yr	81% SLAs	C+	Issue RFP — below
EAP Partners	Employee	\$8.4K/yr	99% SLAs	A	Renew — expand mental
COBRA Admin	COBRA	\$18K/yr	97% SLAs	A-	Renew
FSA/HSA Admin	Consumer	\$24K/yr	92% SLAs	B	Evaluate alternatives
Kincaid Health	Benefits Advisor	Success-fee	Ongoing	A	Annual Stewardship

REGULATORY COMPLIANCE CALENDAR — 2025 ACTION ITEMS

The following compliance calendar tracks all regulatory deadlines, annual filing requirements, and plan administration obligations for the upcoming 12 months. Kincaid Health monitors all regulatory developments and proactively alerts plan sponsors to emerging obligations.

Deadline	Requirement	Regulatory Authority	Status	Owner
January 31	W-2 reporting —	IRS	Filed	Payroll
February 28	Form 1095-C distribution to	IRS / ACA	Mailed	TPA
March 31	Form 1095-C filing with IRS	IRS / ACA	Due Soon	TPA
April 30	RxDC Report — prescription drug	CMS / CAA	In Progress	TPA + PBM
June 1	MHPAEA NQTL Analysis — annual	DOL / ERISA	Not Started	ERISA
July 31	CAA Gag Clause Attestation	CMS / CAA 2021	Filed	Kincaid
September 15	Stop-Loss Renewal — RFP	Internal	Initiate Now	Kincaid
October 1	Open Enrollment begins	Plan Design	Planning	HR
October 15	Form 5500 — extended deadline	DOL / IRS	Prepare	TPA +
November 1	SPD Distribution (if material	ERISA §104	Conditional	TPA
December 31	CAA Machine-Readable File update	CMS / CAA §203	Monthly	TPA

MULTI-YEAR SAVINGS OPPORTUNITY SUMMARY

The following table consolidates all identified savings opportunities from the Employer Health Score™ assessment into a prioritized multi-year action plan. Implementing all recommended actions is projected to generate \$880K-\$1.2M in annual savings within 24 months.

Opportunity	Year 1 Savings	Year 2 Savings	Year 3 Savings	Confidence	Priority
PBM RFP — pass-through	\$60-110K	\$80-130K	\$90-150K	High	P1 —
ER Utilization Reduction	\$95-140K	\$120-180K	\$140-200K	Medium	P1 —
Disease Management	\$57-106K	\$85-140K	\$110-180K	Medium	P1 —
MHPAEA Compliance (avoid	\$0-\$62K/day	Eliminated	Eliminated	High	P1 —
COE Program (surgery)	\$180-280K	\$220-340K	\$250-380K	Medium	P2 — High
Telehealth First Strategy	\$60-90K	\$75-110K	\$90-130K	Medium	P2 — High
Wellness Program Redesign	\$40-65K	\$55-85K	\$70-100K	Low	P2 —
Generic Substitution (GDR	\$38K	\$42K	\$45K	High	P2 — Quick
Reference-Based Pricing	\$0 (pilot)	\$120-200K	\$200-340K	Medium	P3 —
TOTAL OPPORTUNITY	\$530-893K	\$797K-1.2M	\$995K-1.5M	Medium	All in

Projected Year 1 Net Savings (Midpoint)	\$710,000
Projected 3-Year Cumulative Savings	\$2,400,000
Implementation Investment (3-Year Total)	\$175,000
3-Year Net ROI	13.7x

CERTIFICATION & SCORE ATTESTATION

Kincaid Health certifies this Employer Health Score™ was computed using the standardized Kincaid Health scoring protocol (Version 4.2) applied to plan data current as of the assessment date. Scores reflect objective analysis based on 48 individual data points across five weighted domains and do not guarantee specific outcomes. All benchmarking data is sourced from the Kincaid Health National Employer Database (KNED) — a database of 320+ self-funded employer health plans.

This Employer Health Score™ report is prepared for fiduciary review and strategic planning purposes. Plan fiduciaries are encouraged to review this assessment at a scheduled benefits committee meeting and document the plan's response to identified gaps and opportunities in formal committee minutes.

Report Metadata

Report Type	Employer Health Score™ — Full 20-Page Analysis
Composite Score	64/100 — Grade C+
National Percentile	52th
Domains Assessed	5 (Plan Design, Cost Containment, Engagement, Compliance, Rx)
Data Points Evaluated	48
Annual Savings Opportunity (MODELED)	\$79,200
3-Year Net Savings (NPV)	\$225,720
Client	Kincaid Health / Account SHRACK-7742
Report Date	July 22, 2026
Prepared By	Jeremiah Shrack, CEO, Kincaid Health
Confidentiality	Confidential — Strategic Advisory Work Product